



OFFICIAL

Antenatal Clinical Guideline

1. Purpose

- To establish a collaborative relationship between the woman and the health care professionals involved in her care.
- To inform women about the models of care available for pregnancy and birth.
- To obtain baseline recordings of the woman's physical and psychosocial status for comparison at subsequent visits.
- To identify risk factors in pregnancy and make appropriate referrals where necessary.
- To provide support, health education and promote healthy lifestyle habits.
- To guide providers of Antenatal services in the process of providing efficient and safe care.
- To provide culturally appropriate and safe care.
- To prepare women for intrapartum and postnatal care

2. Guidelines

Regular antenatal appointments are an essential part of ensuring the healthiest possible pregnancy. These visits offer personalised information, support and guidance tailored to each woman's unique needs. By attending regular check-ups, potential complications can be identified and addressed early, contributing to better outcomes for both mother and baby. During these appointments, the doctor or midwife will discuss and arrange any necessary screening visits.

3. First visit

If English is not the woman's first language, then an accredited interpreter via telehealth or Telephone Interpreting Service (TIS) must be used.

Avoid use of known persons and relations while addressing to the woman's needs refer to the:

[Interpreter and Translator Guidelines](#)

Initiate discussion with all women that there is no guarantee of a female health professional during their care at Armadale Health service.

At the initial booking visit commence the West Australian Handheld Pregnancy Record (WAHPR) MR220, explaining their use and purpose.¹

Advise the woman to take these records with her:

- To all antenatal appointments and assessment
- To appointments with her General Practitioner (GP)
- On holiday if travelling away
- If she attends the Emergency Department (ED)

At the initial appointment, the following information should be completed:

- Complete MR20 form
- Obtain consent to share Medical Records with GP (AKMR30)
- Complete FDV FDV950
- Check for latex allergy.
- Commence thromboembolic risk assessment (AKMR23.1)
- Generate customised growth charts.
- Complete Obstetric Risk Assessment form (AKMR69.1)
- Complete Safer baby Bundle (SBB) information
- Complete smoking history and perform smoke analyser test and document the levels of Carbon Monoxide for subsequent comparison.

High risk women requiring referral to specialist clinic at a minimum should have the following information documented on the e-referral:

- Reason for referral
- Woman's age, body mass index (BMI)
- Spontaneous or in vitro fertilisation (IVF) pregnancy
- Parity
- Expected date of delivery (EDD)
- Past and current medical, surgical, social history
- Interpreter service required, culturally and linguistically diverse (CALD) patient.
- Abnormal test results (attach result reports)

3.1 Health history

Record the woman's health history including:

- Medical history
- Surgical history
- Gynaecological history
- Obstetric history
- Family history
- Psychological history
- Medications/drugs
- Complementary therapies

3.2 Calculate/Confirm the expected date of delivery

A first trimester ultrasound EDD should be used in preference to the last menstrual period (LMP) if there is a difference of more than 7 days.

When there is a first trimester and second trimester ultrasound available the EDD is determined from the first trimester scan.

In the absence of a first trimester scan, the EDD should be adjusted to the second trimester scan EDD when there is a difference of more than 7 days from the LMP.

4. Clinical examination

4.1 Weight and height

- Measure the height and weight of the woman and calculate their BMI at initial visit.
- Check woman's weight at each visit if BMI >35 or <20 and initiate serial ultrasound scans at 28 weeks, GPO will provide ultrasound requests for remainder of required scans.

NOTE: Ideally BMI should be calculated from a pre-pregnancy weight and height, or at first opportunity during pregnancy.

Formula for Calculating a BMI

$$[\text{Weight in Kilograms} / \text{Height in cm} \times \text{Height in cm}] \times 10,000$$

For example (16.9kg / 105.4cm x 105.4cm) x 10,000 – 15.2

4.2 Blood pressure

Record the woman's blood pressure (BP) at each visit.

- Baseline recording of BP enables comparison and monitoring of BP changes in pregnancy.
- Early baseline measurement will differentiate chronic hypertension in the pregnant woman from gestational hypertension and pre-eclampsia.
- Correct cuff size allows a more accurate BP measurement.

4.3 Breasts

Observe and discuss:

- Previous breastfeeding history, breast surgery or abnormalities and refer to lactation consultant as required.
- Breast changes in pregnancy.

4.4 Pelvic assessment / examination

- Document current cervical screening status.
- The National Cervical Screening Policy (NCSP) recommends that a woman can be safely screened at any time during pregnancy, provided that the correct sampling equipment is used. [Cervical Cancer Screening Guidelines | Cancer Council](#)
- Note any abnormal vaginal discharge and obtain swabs as required.
- Determine if a woman has had female genital mutilation (FGM) performed. Identify the type, provide counselling, and discuss intrapartum management.
- Women with **Grade 2-3 FGM** are referred to the Specialist Obstetric Clinic for review and counselling.

4.5 Cardiovascular system

- Refer women attending with a cardiac anomaly, arrhythmia, or cardiac disease to the Specialist Obstetric and Anaesthetist clinic for review. If a GP has arranged previous cardiac tests request a copy of the results.

5. Laboratory investigations

5.1 Urine analysis

- Instruct women how to perform a urine reagent strip testing for proteinuria/glycosuria.
- Dipstick testing may be useful for indicating pre-eclampsia, urinary tract infection and renal disease.
- Collect a mid-stream urine (MSU) for asymptomatic bacteriuria in early pregnancy if not done by healthcare provider.
- Persistent proteinuria and haematuria may indicate renal disease and further investigations may be warranted.

6. Blood tests

Most antenatal blood tests are typically carried out by the women's healthcare provider and sent along with the antenatal referral. If, at the time of the booking visit, the midwife does not have the full set of documents to complete the WAHPR, all efforts should be made to obtain the missing results. If the results are unavailable, the necessary tests will need to be reordered by the midwife.

Blood tests are recommended for all antenatal patients and should be ordered following counselling and verbal consent. Women should be advised of all results and post-counselling given.

- **Blood group and antibody screen** where the blood group has already been performed it does not need to be repeated, but the antibody screen should be repeated at the beginning of each pregnancy and at 28 weeks.
- **RHD NIPT** – It is recommended that all pregnant women who are determined to be RhD negative are offered RhD NIPT to determine the fetal RhD status. RhD NIPT will be performed at 20-32 weeks gestation (ideally 20-24 weeks) to guide the appropriate use of RhD Ig (Anti-D) at 28-32 weeks and 34-36 weeks gestation.

Women who are RhD negative and decline RhD NIPT or in whom the RhD NIPT result is inconclusive will be managed as if the fetus is RhD positive [AKG Rhesus Negative Women Guideline](#) and receive antenatal RhD Ig.

- **Full blood picture (FBP)**, repeat FBP and ferritin levels at:
 - 28 weeks' gestation.
 - 32 weeks only if on iron.
 - 36 weeks' gestation.
- Iron studies.
- **Hepatitis B.** Women found to be chronic carriers of Hepatitis B should have an assessment of their antigen and viral replicative status with liver functions performed and be referred for Specialist Obstetrician.
- **Hepatitis C.** Women who are Hepatitis C positive should be referred to the Specialist Obstetrician.
- **HIV screening.** The woman must be provided with appropriate counselling as to the limitations of screening for viral infections in pregnancy and the implications of both positive and negative results.

- **Rubella.** Advise woman if non-immune/inadequate immunity she will be offered MMR post birth.
- Varicella
- Syphilis serology
 - Booking,
 - 28 weeks,
 - 36 week or
 - at birth if prior to 36 weeks
- Vitamin D serology
- Chlamydia screening
- Gonorrhoea screening

7. Screening for Gestational Diabetes Mellitus

Screening and management for Gestational Diabetes Mellitus – see clinical guide: [Gestational Diabetes Mellitus and Gestational Diabetes Mellitus requiring pharmacological therapy.](#)

Additional blood tests according to individual clinical situations

- Thyroid function
- Haemoglobinopathy studies

7.1 Group B streptococcus screening.

- Screening for GBS is recommended for women between 35-36 weeks.
- [Group B Streptococcus Guideline](#)

7.2 Methicillin Resistance Staphylococcus Aureus (MRSA) screening

Screen women for MRSA who have been:

- Hospitalised or worked in a hospital outside WA in the previous 12 months.
- A roommate of an active epidemic MRSA carrier during an outbreak occurring in the last 12 months but were not screened prior to discharge.

8. Ultrasound screening

All women should be informed of the availability of screening tests and offered prenatal screening for fetal abnormalities.

Written and verbal information should be provided including advantages and disadvantages, limitations, and consequences of screening.

Screening tests include:

- First trimester screening (FTS), bloods collected 9-13 weeks followed by scan at 11-13 weeks. Review PAPP- A results. If a woman returns a low PAPP- A result (<0.4MoM) commence serial scans at 28 weeks – GPO to review and provide ongoing scan request forms as required.
- Maternal serum screening (MSS), bloods taken at 14-20 weeks (ideally 15-17 weeks).
- Non-invasive pregnancy testing (NIPT), bloods taken at 10-14 weeks.

8.1 Fetal morphology ultrasound

- Offer a fetal morphology ultrasound to all women. Ultrasound screening for fetal anomalies should be routinely offered between 18 and 21 weeks. Anatomy scans are to be arranged by the GP. Advise the woman to arrange to bring a copy of the report to her next antenatal appointment or arrange a copy to be faxed to AHS.
- Review anatomy scan and refer as per [Antenatal Triage of Referral Guideline](#) if any concerns. Document results in HHR.
- Review cervical length, document in HHR and follow pathway of WNHS Guideline [Preterm Birth Prevention: Management](#).
- Serial scan will be required in high-risk pregnancies e.g.
 - Previous still birth
 - Previous small for gestational age (SGA), intrauterine growth restriction (IUGR) babies
 - Low PAPP-A.
 - BMI >35 or <20
 - Medical disorders warranting serial scan.
 - GDM pregnancies
 - Where fundal height will give false reassurance e.g. Fibroids

Arrange a repeat ultrasound for placental location at 32 weeks if a woman has a low-lying placenta (LLP) earlier in pregnancy or at anatomy scan.

8.2 Referral to ultrasound

Indications for a growth scan are:

- First FH measurement below 10th centile (preferably between 26-28 weeks).
- Static growth: no increase in sequential measurements.
- Slow growth: based on sequential measurements, the pattern of growth is not following the slope of the curve.
- Excessive growth: curve linking up plots crossing centiles in an upward direction.

Note that a first measurement above the 90th centile is NOT an indication for a growth scan. A scan would however be indicated if there was clinical suspicion of polyhydramnios or excessive growth on subsequent measurements.

8.3 Serial growth scans for those at increased risk of growth restriction

All women should be assessed at booking for risk factors to identify those at an increased risk of developing fetal growth restriction. Women who are identified at being at an increased risk will need referral to a consultant. The consultant-led team will arrange for serial scans at least every two to three weeks from 26-28 weeks until delivery (earlier gestation or higher frequency if required in individual cases).

8.4 Growth scan requests related to obstetric history:

- Previous birthweight(s) <10th customised centile
- Previous stillbirth

8.5 Growth scan requests related to maternal medical history include:

- Chronic hypertension
- Diabetes

- Renal impairment
- Antiphospholipid syndrome
- Local variances

8.6 Growth scan requests related to current pregnancy:

- Maternal age >40 years
- Maternal smoking
- Drug misuse.
- PAPP-A <0.4MoM
- Fetal echogenic bowel
- Large fibroids
- BMI > 35
- Multiple pregnancy
- Severe pregnancy induced hypertension.
- Pre-eclampsia
- Unexplained antepartum haemorrhage (APH)
- Concerns related to growth measurements, as listed above.

All ultrasound reports must be acknowledged, and actions taken documented either directly in the case notes or on the ultrasound report directly, which is then kept in the woman's case file.

Palpate for presentation from 36 weeks gestation. Abnormal presentation needs to be confirmed on ultrasound, with a review and plan made in the AAU.

9. Psychological assessment

Perform Edinburgh Post Natal Depression Scale (EPDS) assessment at booking visit.

Refer the woman (with their permission) to the Perinatal Mental health Liaison Nurse if:

- the EPDS score is 13 or above.
- a woman scores 1, 2 or 3 of EPDS to question 10, assess her current safety and the safety of other children in her care and, acting according to clinical judgement, seek advice from clinic co-ordinator.
- the anxiety scale is more than 6 (Q3+Q4+Q5 = 6).
- there are current mental health disorders or significant symptoms.
- a personal history of diagnosed mental disorder is present.
- the woman is at risk of harming herself (or others) due to psychiatric disturbances.

If you have serious concerns about the woman's psychological wellbeing, contact the MLN or Social work and escort safely to ED for assessment.

Repeat the Edinburgh Postnatal Depression Scale after 32 weeks.

10. Family and domestic violence (FDV) screening

Important: Ensure the woman is alone before completing the screening process

- Explain to all women that asking about domestic violence is a routine part of antenatal care and enquire about all women's exposure to domestic violence.
- Screen the woman when she is alone, tailoring the approach to her individual situation.
- Document and file the result of FDV950 in the Medical Records, not in the HHR.
- Repeat the FDV screening tool in the third trimester.

11. Diet

Record and provide advice about any dietary practices which may impact the pregnancy.

- Advise women that taking vitamins A, C or E supplements are no benefit in pregnancy and may cause harm.
- Advise women to take an iodine supplement of 150 micrograms each day. Women with pre-existing thyroid conditions should seek advice from their medical practitioner before taking a supplement.
- Women having a vegetarian diet or with barriers for gastric absorption may require additional nutritional supplements in pregnancy.

12. Oral health

Advise women to have oral health checks and treatment, if required, as good oral health protects a woman's health and treatment can be safely provided during pregnancy.

13. Tobacco smoking assessment

All women are to receive carbon monoxide screening at their first antenatal appointment for (CO) exposure, and for those that smoke repeat screening at every appointment and commence serial scans from 28 weeks.

Refer women who smoke, vape, or chew tobacco to the quit line as indicated [Quit Smoking & Vaping: Get Expert Cessation Tips & Help | Quit](#)

14. Alcohol use

Assess alcohol use prior to pregnancy and during pregnancy completing the Audit C tool in the NWPHR and commence serial scans from 28 weeks if alcohol consumption present.

15. Antenatal Models of Care

Explain the options of antenatal care:

- Antenatal Clinic
- Midwifery Group Practice (MGP)
- GP own
- GP Public
- Specialist Obstetric Clinic
- Supporting those at risk (STAR)
- Next birth after caesarean (NBAC)

- Supporting women with additional nutritional needs (SWANN)

16. Initiate parent education

- Follow the education checklist in the WAHPR.
- Discuss the hospital-based parent education classes and advise on how to book in.
- Discuss National Preterm Birth Prevention Collaboration initiative, and the avoidance of birth before 39 completed weeks of gestation unless there is a medical or obstetric reason.

17. Subsequent antenatal visits (at each appointment)

A standard schedule of antenatal care is provided within the HHR.

Refer to individualised guidelines for specialised clinics and their schedule for antenatal visits.

17.1 Assess maternal health and well-being.

Assess if the woman has any abnormal health symptoms e.g., headaches, epigastric pain, vomiting, visual disturbances. These symptoms may be associated with pre-eclampsia and further investigations, or evaluation may be required.

- BP.
- Urinalysis screen for proteinuria and glycosuria, educate woman to self-screen.
- Oedema assessment. Identify and document oedema including the site and degree.
- Vaginal discharge, note at each visit:
 - If any vaginal bleeding has occurred since the last visit.
 - Signs of vaginal infection.
 - Signs of premature rupture of membranes.

Women should be informed that increased vaginal discharge is a normal physiological change in pregnancy, however if they experience an itch, soreness, offensive smell, or pain with voiding they should inform the midwife or doctor.

17.2 Fetal assessment

- At each visit perform abdominal palpation and note fetal movements.
- Assess fundal height, growth, and adequate amniotic fluid as appropriate.
- Standardised Fundal Height (SFH) should be measured in cm and documented on the Customised Growth Charts chart every 2-3 weeks from 26-28 weeks for low-risk women. The Chart must be generated for women at the time of booking and stapled onto page 27 of the HHR.
- Not all pregnancies are suitable for primary surveillance by fundal height measurement and require ultrasound biometry instead. In most instances, these pregnancies fall into the following categories:
 - Fundal height (FH) measurement unsuitable/inaccurate e.g., large fibroids, BMI ≥ 35 , multiple pregnancy.
 - Pregnancy considered increased risk requiring serial ultrasound e.g., pre-existing diabetes.
- Enquire about normal fetal movements.
- Monitor the history of fetal activity at each visit.
- Auscultation of the fetal heart with a fetal doppler.

- If concerns are identified during an antenatal appointment which necessitate an urgent obstetric review, the clinic midwife shall contact the Antenatal Assessment Unit (AAU) and liaise directly with the midwifery coordinator
- For women requiring a non-urgent review by the GPO in the antenatal clinic within the next seven days, the clinic midwife shall liaise with the antenatal clinic coordinator, who will arrange an appointment through the GPO Practice Manager.

17.3 At 40 weeks in either Midwifery or GP clinic:

- Discuss induction of labour (IOL)
- Organise 41 weeks scan for fetal wellbeing (AFI and Doppler)
- Book cardiotocography (CTG) in AAU at 41 weeks following scan.

17.4 Advise women at 41 weeks AAU assessment the following will be discussed:

- Review CTG
- Review scan report.
- Offer vaginal examination to assess Bishop Score
- Offer date and time for IOL.

18. Membrane sweeping

- A Cochrane review concluded there is little justification for performing routine sweeping of membranes for women near term (37 to 40 weeks of gestation) in an uncomplicated pregnancy.

Membrane sweeping can be discussed and offered to women from 40+0 weeks, following informed consent.

It can be repeated as clinically appropriate and in accordance with local protocols until spontaneous labour occurs or induction of labour becomes necessary.

If a woman requests membrane sweeping before 39+6 weeks, she should be advised that this procedure is not provided in the antenatal or GPO clinic.

19. Increased surveillance for women aged 40 years or over

All women who are age 40 years or over require increased surveillance from 38 weeks gestation.

- At 38 weeks gestation these women require a twice weekly CTG and one ultrasound scan.
- At 39 weeks gestation they require twice weekly CTG.
- At 40 weeks gestation they require a CTG, ultrasound scan and a discussion regarding induction of labour.
- Induction of labour is recommended from 40-41 weeks gestation.

20. Increased surveillance of woman from culturally and linguistically diverse (CALD) background

Risks related to changed environment, psychological aspects, dietary changes need to be assessed and kept in mind. Use of interpreter services should be done all the time if English is

not the first language and woman has difficulty understanding. Avoid use of relatives or friends for interpreter purposes. Book an ultrasound scan for fetal wellbeing and estimated fetal weight at 38 weeks. Plot estimated fetal weight on customised GROW Chart and review the growth and wellbeing. Refer to AAU if any deviation from normal.

21. Increased surveillance in IVF pregnancies and consideration of IOL between 39-40 weeks.

There is increased risk of perinatal morbidity and mortality, prematurity, IUGR and PET in IVF pregnancies. Consider increased surveillance for fetal well-being.

22. Policy context

The following documents are required to give effect to this policy:

- HDWA
 - [Clinical Handover Policy](#)
 - [Recognising and Responding to Acute Deterioration Policy](#)
- EMHS
 - [Patient Identification and Procedure Matching \(EMHS: 21\)](#)

23. Supporting information

The following documents support the implementation of this policy:

- AKG
 - [Antenatal: Triage of Referral Guidelines](#)
 - [Gestational Diabetes Mellitus and Gestational Diabetes Mellitus Requiring Pharmacological Therapy Guideline](#)

24. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Clinical Governance - Action 1.5
- Standard 2 – Partnering with Consumers - Action 2.10
- Standard 5 – Comprehensive Care - Action 5.3
- Standard 6 – Communicating for Safety - Action 6.5, 6.7

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26. Document owner

Director Clinical Services

Enquiries relating to this guideline may be directed to:

Title: Head of Department
 Department: Obstetrics and Gynaecology
 Email: sangeeta.mallabhat@health.wa.gov.au

Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V6.2	31 03 2026	14 03 2028	Minor review to align with the new GDMi policy document.

27. Authorisation

This guideline has been authorised and issued by:

Authorised by	Dr Tania Strickland – Director Clinical Services
Authorisation date	30 03 2026
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Appendix 1: Minimum schedule for visits for antenatal care

WKS	ANC/MGP/MAPS Risk: A/B	ANC/MGP/MAPS Risk: C	GP/GPO/EM shared care model
14			
18	MIDWIFE	MIDWIFE	GP/GPO/EM
20	MIDWIFE	SPEC OBS	GP/GPO/EM
24	MIDWIFE	MIDWIFE	MIDWIFE
28	GPO	SPEC OBS	GPO/EM
32	MIDWIFE	MIDWIFE	GPO/EM
34	MIDWIFE	SPEC OBS	GPO/EM
36	MIDWIFE OR GPO	SPEC OBS	GPO/EM
38	MIDWIFE	MIDWIFE	MIDWIFE
40	MIDWIFE	SPEC OBS IF REQUIRED	GPO/EM
41	AAU	SPEC OBS IF REQUIRED	AAU

BOOKING VISIT 12-16 Weeks Review Previous notes. ➤ Complete HHR. ➤ Enter all results. ➤ Obtain Maternal history. ➤ Screen FDV ➤ Audit C ➤ CO monitor ➤ Discuss fetal movements. ➤ Pertussis/Flu vaccination. ➤ Complete MR20 ➤ Medical release. ➤ STORK/GROW chart. ➤ Refer SW/MLN ➤ Earl GTT if high Risk Review Obstetric risk Triage and refer	20 weeks Review Anatomy scan Document results in HHR Arrange Early GTT if high risk Record Maternal fetal observations Review Risk and Refer if indicated	30-32 Weeks Document Results Arrange follow up Scan if LLP Follow up USS and plot on GROW chart Discuss fetal movements. Document Reassess EPDS/DV assessment Review Risk management	38 weeks Discuss fetal movements document. Review all results and document. Complete birth plan/consents Review USS results and plot Update Neonatal records Review risk management plan
	24-26 Weeks Document results from Anatomy Scan Information for GTT Form for FBC Fe studies/GTT/Group and antibodies if Rhesus neg Plot fundal height on GROW chart Arrange USS 28/40 if risk factors. Review risk /follow pathway/Refer	34 weeks Discuss fetal movements. Discuss birth Plan Consents for Vitamin K Hepatitis B Information on SIDS Information for GBS screening Update results on HHR Review risk and refer	40 Weeks midwife clinic and GP Obs clinic Update results. Discuss fetal movements and document. Discuss IOL Book an AAU appointment for biweekly CTG commencing 41 weeks onwards and alternate day CTG commencing 42 weeks onwards
	28 Weeks GP OBS Review Follow up Blood results Administer Anti D/Pertussis/Flu/RSV/vaccine Review GTT Plot fundal height on GROW chart Review BMI Discuss fetal movements: document Review risk document on MR (Risk Management)	36 Weeks GPO or Midwife Obtain GBS with consent Administer Anti D Blood form for FBC/Fe studies Review GROW Chart/Discuss fetal movements Arrange USS if risk factors Update Risk management	41 weeks AAU appointment: Review fetal movements, book biweekly CTG Book alternate day CTG if ≥ 42 weeks. Book IOL from 41 week onwards depending on Bishop score, CTG and scan findings. Discuss/ inform re IOL and obtain Consents

Refer if risk change: update management plan